Vedolizumab (Entyvio) IV and SC

Policy number	LHP-RX-114	**Version**	3
Effective date	07/01/2026	**Last reviewed**	06/10/2026
Applies to	Medical benefit and pharmacy benefit	**Next review**	06/2027
Line of business	Commercial (fully insured and self-funded)	**Approved by**	P&T Committee

Policy statement

Lakeshore Health Plan considers vedolizumab medically necessary when the criteria in this policy are met. Requests that do not meet these criteria are not medically necessary. This policy applies to the intravenous vial and the subcutaneous prefilled pen. Benefit assignment for each form follows the Lakeshore Health Plan Specialty Drug List.

Background

Vedolizumab is a gut-selective integrin receptor antagonist (alpha-4 beta-7) for adults with moderately to severely active ulcerative colitis or Crohn's disease. It is supplied as a 300 mg single-dose vial for intravenous infusion and a 108 mg/0.68 mL single-dose prefilled pen for subcutaneous injection. Labeled induction is 300 mg intravenously at weeks 0 and 2. At week 6, patients may continue intravenous therapy every 8 weeks or switch to 108 mg subcutaneously every 2 weeks.

1. Covered indications

Moderately to severely active ulcerative colitis or Crohn's disease in adults.

2. Definitions

“Conventional therapy” means an aminosalicylate, a systemic corticosteroid, or an immunomodulator (azathioprine, 6-mercaptopurine, methotrexate).

“Preferred biologic” means an infliximab product (including biosimilars) or an adalimumab product (including biosimilars). Appendix A lists the products included in each group.

3. Initial approval criteria

All of the following:

3.1 Diagnosis of moderately to severely active ulcerative colitis or Crohn's disease, supported by clinical or endoscopic findings.

3.2 Prescribed by, or in consultation with, a gastroenterologist.

3.3 Inadequate response, intolerance or contraindication to at least one conventional therapy.

3.4 Step therapy: trial of at least one preferred biologic with inadequate response, loss of response or intolerance, unless contraindicated. Documentation must name the agent and the outcome.

Each criterion is reviewed against the records submitted with the request. A criterion is met only when a record supports it; the reviewer cites the record used.

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4. Approval duration

IV induction: up to 2 doses. SC maintenance: 6 months.

5. Renewal

Documented clinical response; 12 months.

6. Quantity limits

Pen: 2 per 28 days. IV: 300 mg per dose.

7. Not covered

Use together with another biologic or targeted small molecule for the same condition.

8. Site of care

IV doses at an in-network infusion center or hospital outpatient department.

9. Dosing reference (ulcerative colitis)

Phase	Form	Labeled dose	Covered under this policy
Induction	IV 300 mg vial	300 mg IV over 30 minutes at weeks 0 and 2	Up to 2 doses (section 4)
Maintenance	SC pen 108 mg	108 mg SC every 2 weeks, starting at week 6	6 months, then renewal (sections 4, 5)
Maintenance	IV 300 mg vial	300 mg IV at week 6, then every 8 weeks	Requires a separate request

Per the label, discontinue therapy in patients who show no evidence of therapeutic benefit by week 14.

10. Documentation to submit

Submit records that support each criterion. Requests without them may receive a request for information.

- A clinic note documenting current disease activity, dated within 90 days of the request.
- For ulcerative colitis, a colonoscopy or flexible sigmoidoscopy report, or a Mayo score, dated within 90 days.
- The prescriber's specialty, or the name of the consulting gastroenterologist.
- Conventional therapy tried: agent, dates and outcome.
- Preferred biologic tried: agent, dates and outcome, or the contraindication.
- For IV doses, the site of care.

11. Coding

Code	Description
HCPCS J3380	Injection, vedolizumab, intravenous, 1 mg (300 units per 300 mg dose)
ICD-10-CM K51.00 to K51.919	Ulcerative colitis
ICD-10-CM K50.00 to K50.919	Crohn's disease

The pen is a pharmacy-benefit product and is not billed with a HCPCS code.

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Appendix A. Preferred biologic products (generic names)

Group	Products included
Infliximab products	infliximab, infliximab-dyyb, infliximab-abda, infliximab-axxq
Adalimumab products	adalimumab, adalimumab-adaz, adalimumab-atto, adalimumab-adbm, adalimumab-aaty, adalimumab-fkjp, adalimumab-bwwd, adalimumab-aqvh, adalimumab-aacf, adalimumab-ryvk

A trial of any listed product satisfies criterion 3.4 when the outcome is documented. Products are listed by nonproprietary name only; a record that names a product by brand refers to the same product.

Appendix B. Reviewer worksheet for criteria 3.1 to 3.4

Reviewers record a result for each criterion and the record that supports it. Any criterion marked not met or missing is explained in the determination.

Criterion	What the record must show	Result	Supporting record and page
3.1 Disease activity	Moderately to severely active ulcerative colitis or Crohn's disease; clinic note, endoscopy report or Mayo score	[ ] Met [ ] Not met [ ] Missing	
3.2 Prescriber	Gastroenterologist, or consultation with one	[ ] Met [ ] Not met [ ] Missing	
3.3 Conventional therapy	At least one aminosalicylate, systemic corticosteroid or immunomodulator, with the outcome	[ ] Met [ ] Not met [ ] Missing	
3.4 Preferred biologic	At least one infliximab or adalimumab product (Appendix A), naming the agent and the outcome, or a contraindication	[ ] Met [ ] Not met [ ] Missing	

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References

1. Vedolizumab prescribing information. U.S. Food and Drug Administration label, revised 2026.

2. Rubin DT, et al. ACG clinical guideline update: ulcerative colitis in adults. Am J Gastroenterol. 2025.

3. Singh S, et al. AGA living clinical practice guideline on pharmacological management of moderate-to-severe ulcerative colitis. Gastroenterology. 2024.

4. Lakeshore Health Plan Specialty Drug List, effective 07/01/2026.

Policy history

Version	Effective	Summary of changes
1	01/01/2024	New policy for intravenous vedolizumab.
2	07/01/2025	Added the subcutaneous pen. Added quantity limits.
3	07/01/2026	Combined IV and SC criteria. Added definitions of conventional therapy and preferred biologic. Step therapy documentation must name the agent and the outcome. Added Appendices A and B.

This policy is used to make coverage decisions and is not medical advice. Treating providers are responsible for clinical decisions. Coverage depends on the member's benefit plan, and the plan document governs where it differs from this policy.

Contacts

Medical benefit prior authorization and appeals: provider services (800) 555-0131; UM fax (800) 555-0138.

Pharmacy benefit prior authorization: Lakeshore Rx Solutions pharmacy help desk (800) 555-0136; PA fax (800) 555-0137.

Clinical policies and the specialty drug list: lakeshorehealthplan.example/policies

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